14.1 Hypovolemic Shock*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I feel cold and thirsty." **Objective** • BP 80/50 mmHg, HR 132/min • Cold, clammy skin; capillary refill 4 seconds • Urine 10 mL/hour • Known blood or fluid loss	**ACTUAL** Inadequate fluid volume related to acute blood or fluid loss as evidenced by hypotension, tachycardia, poor perfusion and oliguria.	• MAP of 65 mmHg or more and urine at least 0.5 mL/kg/hour within 2 hours.	• Call for help and stop external bleeding with direct pressure. • Insert two large-bore cannulas. • Give warmed fluids and blood products as prescribed. • Check BP, HR, SpO_2 and consciousness every 5–15 minutes. • Insert a urinary catheter and measure output hourly.	• Achieved when MAP and urine output reach target. • If not: prepare for surgical control of the hemorrhage.
Subjective • Drowsy **Objective** • Confused, slow to respond • SpO_2 92%	**ACTUAL** Acute confusion related to reduced cerebral perfusion as evidenced by drowsiness and slowed responses.	• Alert and oriented as perfusion improves.	• Check GCS every 15–30 minutes. • Give oxygen to the target SpO_2. • Keep him flat with the legs raised unless contraindicated.	• Outcome reached once he is alert.
Subjective *No specific complaint* **Objective** • Massive transfusion • Cold fluids given • Exposed during resuscitation	**RISK** Risk for decreased body temperature as evidenced by risk factors: blood loss, cold IV fluids and exposure during resuscitation.	• Core temperature stays above 36 °C.	• Run blood and fluids through a warmer. • Apply warm blankets or a forced-air warmer. • Measure temperature every 30–60 minutes.	• Considered met while temperature stays normal.

*References for this care plan: [2], [35], [64], [128]. Full entries are listed in the References section of the CPMN book.